

BUNDIBUGYO VIRUS DISEASE OUTBREAK

Democratic Republic of the Congo | Uganda | France

Weekly External Situation Report 18, Data as of 13 September 2026

Key Figures at a Glance (Democratic Republic of the Congo)

7 Provinces	6 Provinces	1 726	66.3%	27 905
62 Health Zones Affected	49 Health Zones Active Transmission*	Cases Recovered	Bed Occupancy	Contacts under follow up
905 Active Cases in Isolation	167 Health Worker Cases	47 Health Worker Deaths	1 366 Bed Capacity	26 Testing Laboratories

Note: Health zones with active transmission have reported at least one case in the last 21 days. Active cases in isolation include both suspected and confirmed cases.

Summary

Country	New Confirmed Cases (Last 7 days)	New Confirmed Deaths (Last 7 days)	Confirmed Cases (Total)	Confirmed Deaths (Total)	Probable Cases	Probable Deaths	Total Cases (Confirmed & Probable)	Total Deaths (Confirmed & Probable)	CFR (%) Confirmed & Probable	Outbreak Status
Democratic Republic of the Congo	572	284	7 258	3 510	-	-	7 258	3 510	48.4%	Active
Uganda	0	0	20	2	1	1	21	3	14.3%	Ended
France	0	0	1	0	0	0	1	0	0.0%	Ended

Note: Only confirmed and probable cases are reported.

Event description

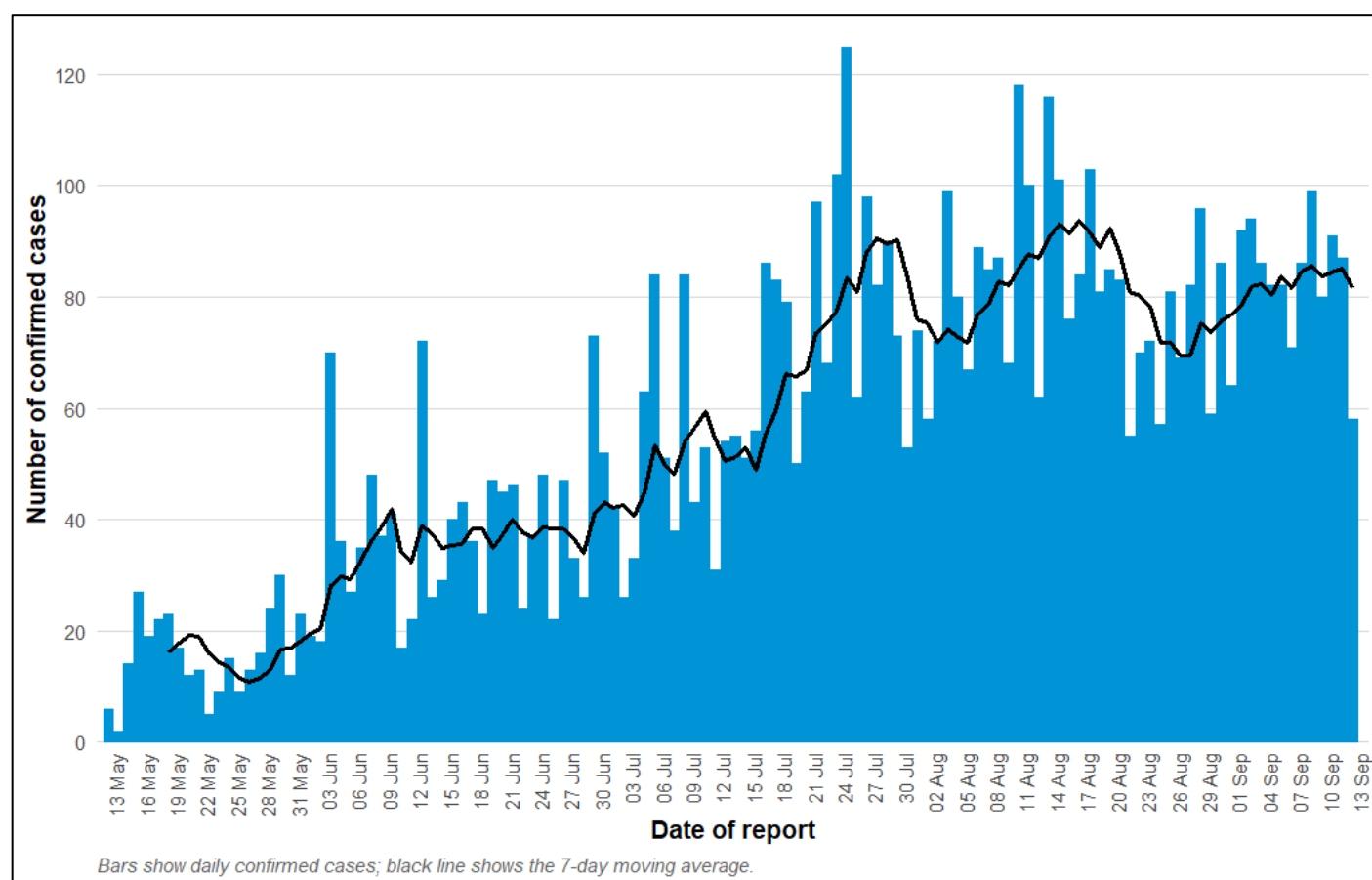
Democratic Republic of the Congo

The Bundibugyo virus disease (BVD) outbreak in the Democratic Republic of the Congo has expanded to a seventh province, with the first confirmed case reported in Bulu Health Zone, Sud-Ubangi, a province bordering the Central African Republic and the Republic of the Congo. This latest geographic expansion heightens concern about further spread towards international borders, while transmission within the country remains increasingly heterogeneous across affected provinces and health zones. Since [External Situation Report #17](#), a further 572 confirmed cases and 284

confirmed deaths have been reported, bringing the cumulative total to 7258 confirmed cases, including 3510 deaths [case fatality ratio (CFR) 48.4%], as of 13 September 2026. Ituri remains the principal focus but its relative contribution continues to decline, accounting for 78.0% of cumulative confirmed cases, while transmission continues to increase in Nord-Kivu and persists in Haut-Uélé. The outbreak now affects 62 health zones across seven provinces, with Bulu Health Zone in Sud-Ubangi being the latest affected.

At the national level, daily incidence remains high and fluctuating, with the seven-day moving average declining from its mid-August peak, followed by a modest rebound in early September. This national pattern masks increasingly divergent provincial trajectories. Ituri continues to decline from its mid-August peak but remains at a high level, while Nord-Kivu is experiencing a sharp and sustained increase, reaching its highest incidence since the start of the outbreak and increasingly driving the national trajectory. Transmission in Haut-Uélé remains sustained but has declined from its August peak. Tshopo shows a recent increase from a low baseline, while Bas-Uélé continues to report sporadic transmission, and no recent transmission is evident in Sud-Kivu. Overall, the epidemic is becoming more geographically heterogeneous, with declining transmission in Ituri and Haut-Uélé occurring alongside rapid intensification in Nord-Kivu, renewed activity in Tshopo and continued geographic expansion to new areas.

Figure 1. Daily national trend in confirmed Bundibugyo virus disease cases, with seven-day moving average, by date of report, Democratic Republic of the Congo, as of 13 September 2026



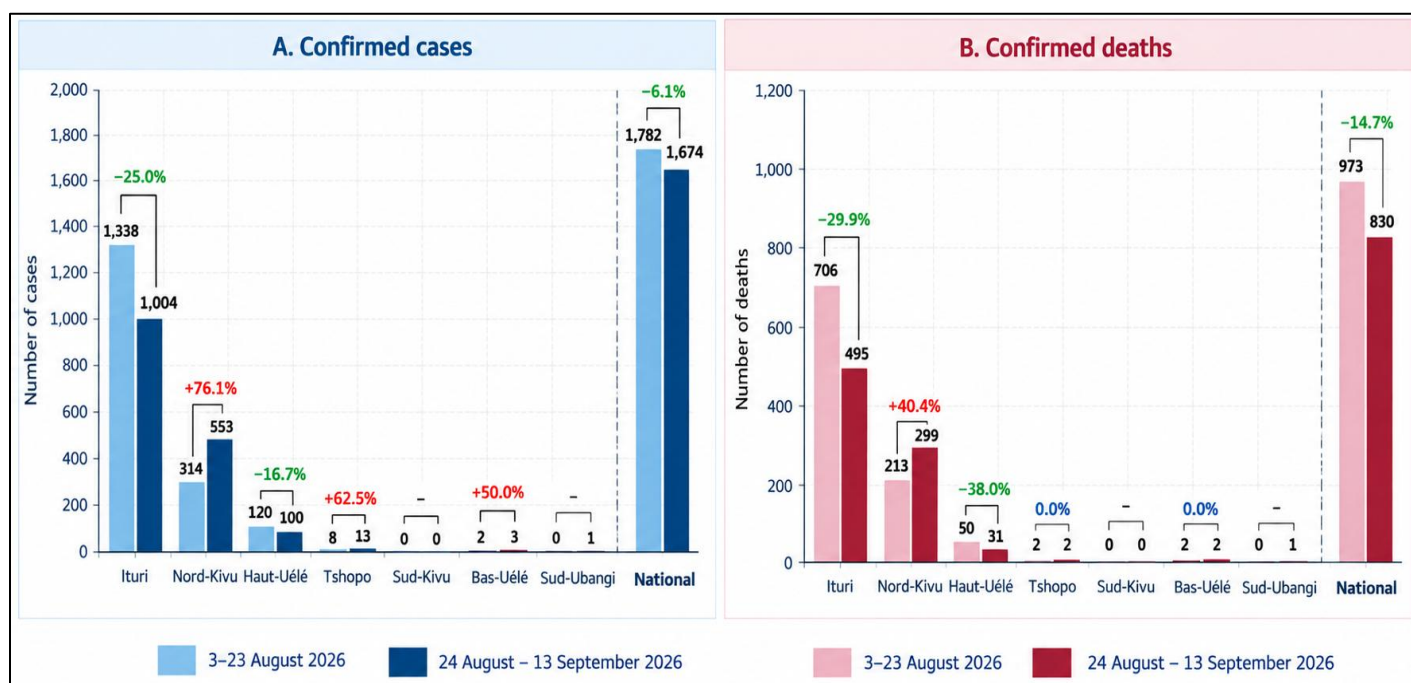
During the most recent 21 days (24 August – 13 September 2026), 1674 confirmed cases were reported nationally. Compared with 1782 cases during the preceding 21-day period (3 – 23 August 2026), this represents a decrease of 108 cases (–6.1%). Reported cases declined by 25.0% in Ituri, from 1338 to 1004, and by 16.7% in Haut-Uélé, from 120 to

100. In contrast, cases increased sharply by 76.1% in Nord-Kivu, from 314 to 553, while Tshopo increased from 8 to 13 cases and Bas-Uélé from 2 to 3 cases. One case was also reported in the newly affected province of Sud-Ubangi during the latest period. Consequently, Ituri's contribution to newly reported cases fell markedly from 75.1% to 60.0%, while Nord-Kivu's contribution nearly doubled from 17.6% to 33.0%; Haut-Uélé's contribution declined slightly from 6.7% to 6.0%. Overall, the modest 6.1% national decline masks a pronounced geographic redistribution of transmission, with the substantial decline in Ituri increasingly offset by rapidly intensifying transmission in Nord-Kivu and continued geographic expansion.

During the same period, 830 confirmed deaths were reported nationally, compared with 973 deaths during the preceding 21 days, representing a decrease of 143 deaths (–14.7%). The national decline was driven largely by Ituri, where reported deaths decreased from 706 to 495 (–29.9%), while deaths also declined in Haut-Uélé, from 50 to 31 (–38.0%). In contrast, deaths increased substantially in Nord-Kivu, from 213 to 299 (+40.4%). Consequently, Ituri's contribution to newly reported deaths fell markedly from 72.6% to 59.6%, while Nord-Kivu's contribution increased from 21.9% to 36.0%; Haut-Uélé's contribution declined from 5.1% to 3.7%. Tshopo and Bas-Uélé each reported two deaths during the latest period, while one death was reported in Sud-Ubangi.

Overall, the national reductions in both cases (–6.1%) and deaths (–14.7%) were driven predominantly by declining transmission and mortality in Ituri and mask sharply divergent provincial trajectories. In particular, the simultaneous increases in cases (+76.1%) and deaths (+40.4%) in Nord-Kivu indicate substantial intensification of the outbreak there, with the province now accounting for one-third of newly reported cases and more than one-third of newly reported deaths nationally.

Figure 2. Bundibugyo virus disease cases and deaths by province across consecutive 21-day periods, 03 – 23 August 2026 compared with 24 August – 13 September 2026, Democratic Republic of the Congo

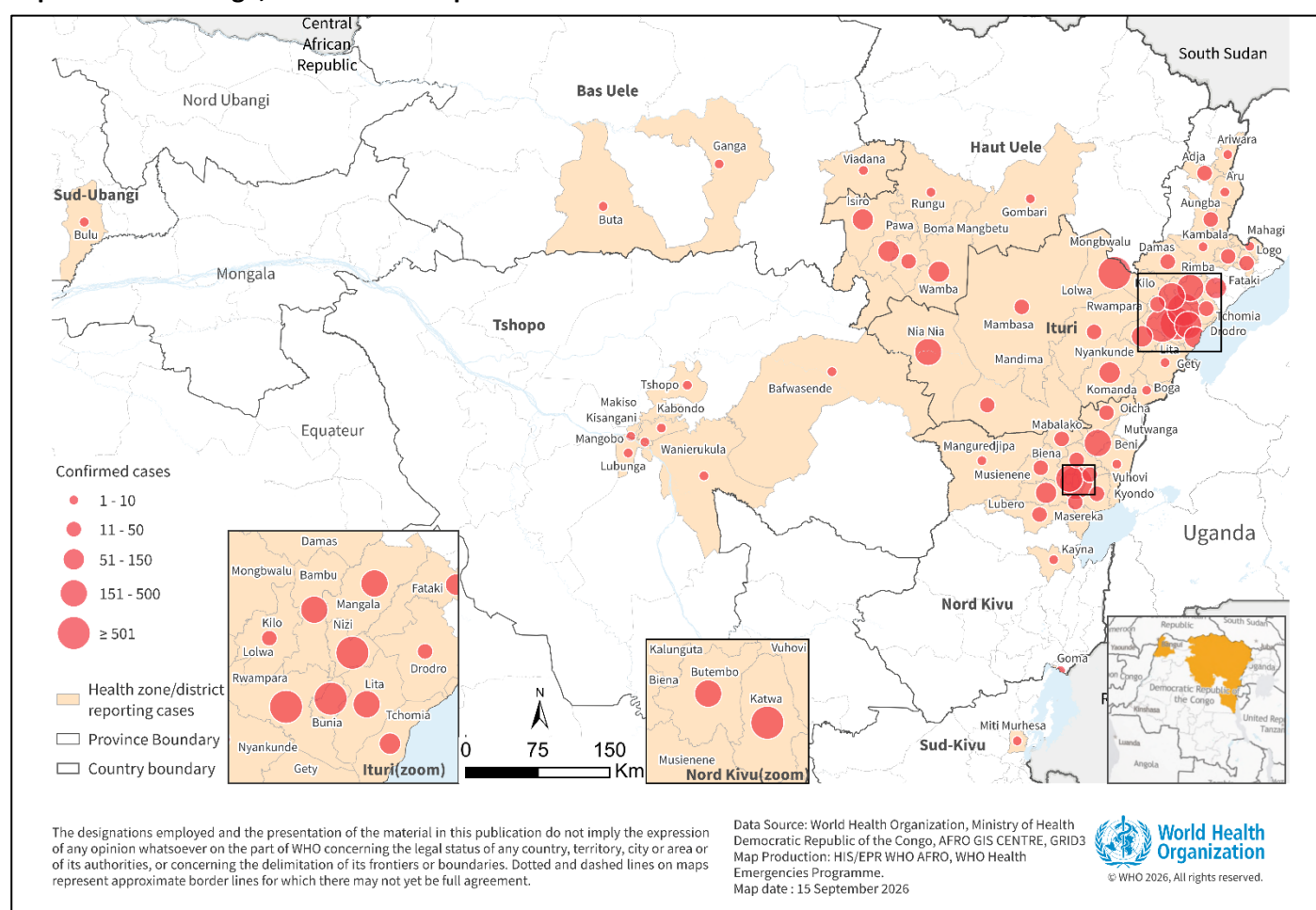


Additionally, during the most recent 21 days (24 August – 13 September 2026), 49 of the 62 affected health zones (79.0%) reported at least one new confirmed case, while 13 (21.0%) reported no new cases. Health zones reporting no cases were Adja, Ariwara, Aungba, Kambala and Mahagi in Ituri; Goma in Nord-Kivu; Gombari in Haut-Uélé; Lubunga,

Tshopo and Wanie-Rukula in Tshopo; Miti-Murhesa in Sud-Kivu; and Buta and Viadana in Bas-Uélé.

Provincial trends also mask important differences between health zones. In Ituri, where cases declined by 25.0% overall, several health zones continued to increase, notably Komanda (22 to 76; +245.5%), Mangala (92 to 116; +26.1%), Lita (56 to 65; +16.1%) and Mongbwalu (50 to 61; +22.0%). In contrast, major transmission areas declined substantially, including Bunia (384 to 285; -25.8%), Nizi (210 to 120; -42.9%), Rwampara (258 to 106; -58.9%) and Nia-Nia (89 to 46; -48.3%). Nord-Kivu shows the opposite pattern, with broad-based intensification rather than an increase confined to a few hotspots. Cases increased in Katwa (162 to 175; +8.0%), Beni (53 to 111; +109.4%), Butembo (51 to 101; +98.0%) and Musienene (27 to 46; +70.4%), alongside transmission in newly or recently affected health zones. Haut-Uélé declined overall, but this also concealed a marked increase in Pawa, from eight to 38 cases (+375%), while Isiro and Wamba declined.

Figure 3. Geographical distribution of cumulative confirmed cases of Bundibugyo virus disease in the Democratic Republic of the Congo, data as of 13 September 2026



Weekly confirmed deaths peaked at 364 during 10 – 16 August, before declining to 302 and 270 over the following two weeks. Deaths subsequently increased to 276 during 31 August – 6 September and further to 284 during 7 – 13 September. The composition of mortality, however, shifted in the latest week. Community deaths decreased from 207 to 193 (-6.8%), while deaths in treatment facilities increased from 69 to 91 (+31.9%). Consequently, the proportion of deaths occurring in the community declined from its peak of 75.0% to 68.0%, although it remains substantially higher than in most earlier weeks.

The persistence of high community mortality may reflect multiple barriers along the pathway to care, including delayed detection and notification, delayed referral or transfer to treatment facilities, limited recognition of illness or perceived severity, geographic and transport barriers, care-seeking outside formal health facilities, and community acceptance or trust. These factors may result in patients reaching treatment facilities late or dying before referral can be completed. The continued predominance of community deaths, despite expanding treatment capacity, therefore suggests that increasing bed capacity alone may be insufficient and reinforces the need to strengthen early case detection, rapid referral and community-level pathways to timely care.

Figure 4. Weekly number of confirmed Bundibugyo virus disease deaths by place of death, Democratic Republic of the Congo, epidemiological week 26 – 37, 2026

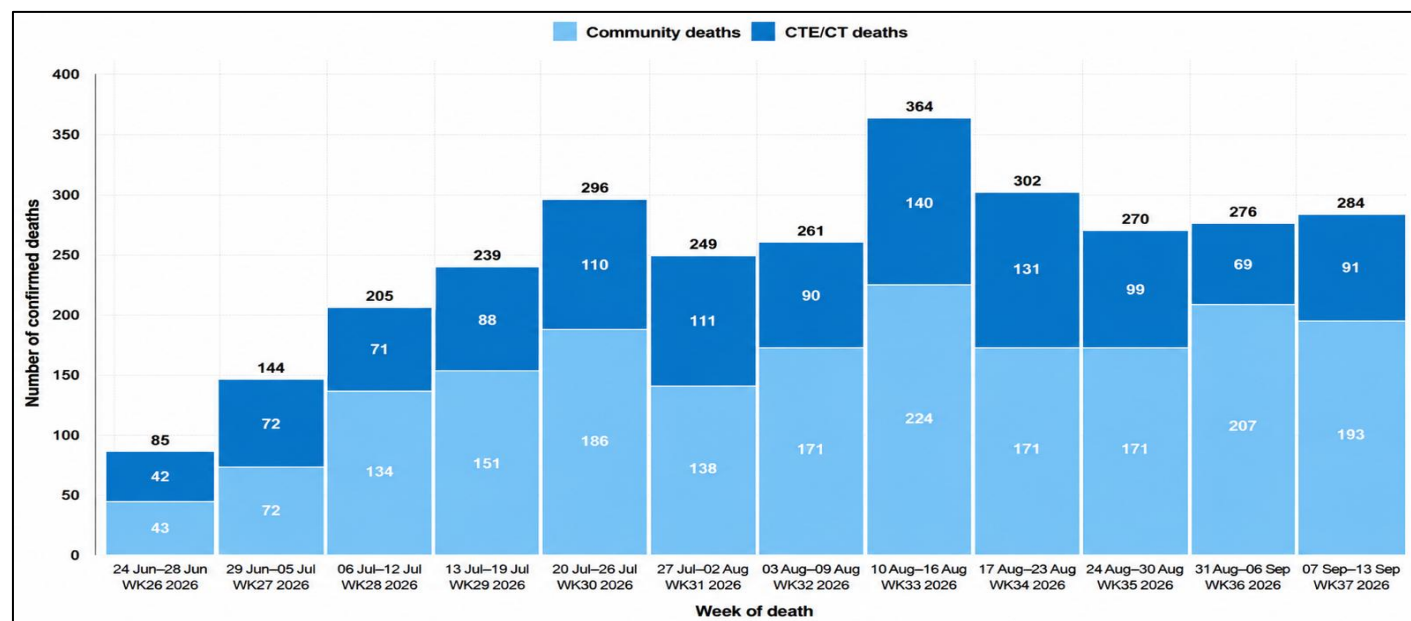
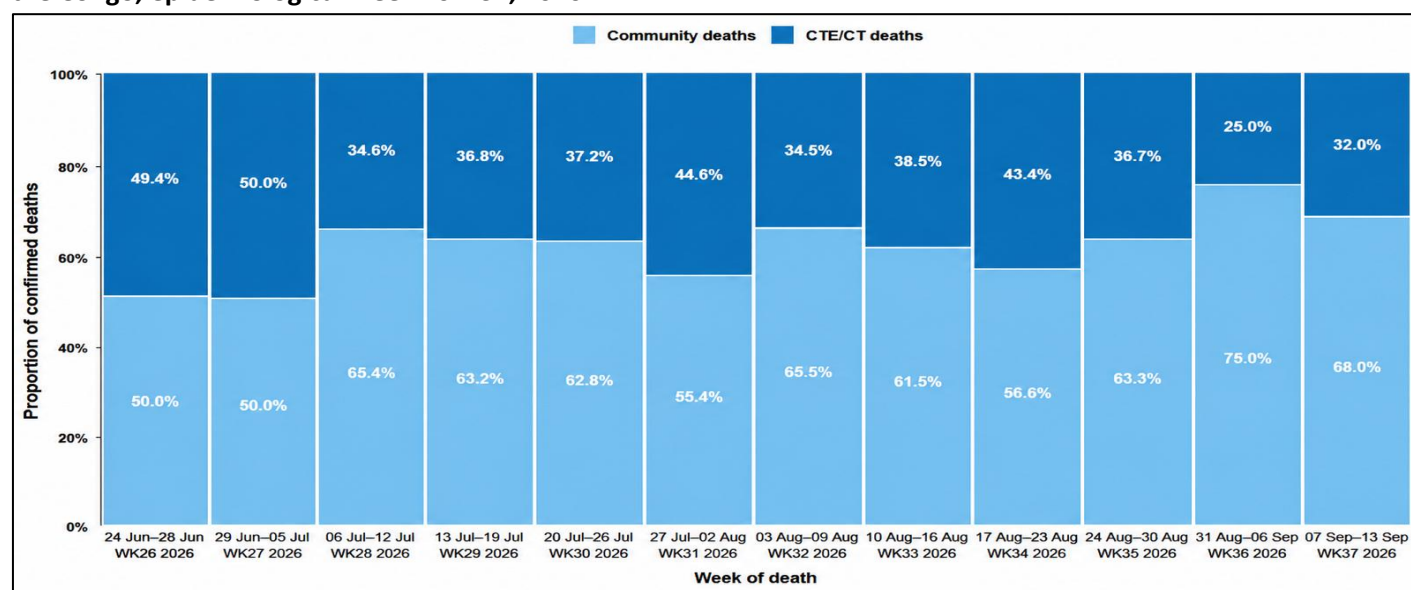


Figure 5. Weekly proportion of confirmed Bundibugyo virus disease deaths by place of death, Democratic Republic of the Congo, epidemiological week 26 – 37, 2026



Risk Assessment

The risk of further spread remains very high within the Democratic Republic of the Congo. This assessment reflects sustained transmission, continued geographic expansion, high mortality, population mobility, insecurity and persistent response challenges. A two-week invasion-risk forecast has identified 20 currently unaffected health zones at elevated risk of transmission. In descending order of predicted risk, these are Rethy, Watsa, Nyarambé, Biringi, Dungu, Makoro, Karisimbi, Angumu, Rutshuru, Nyiragongo, Niangara, Linga, Kiotshe, Bafwagbogbo, Alimbongo, Jiba, Poko, Rwanguba, Kamango and Kibirizi. The highest predicted risks are concentrated in Ituri and Haut-Uélé, reinforcing the need for risk-based preparedness and readiness measures.

The risk is considered high for neighbouring countries sharing land borders with the Democratic Republic of the Congo and low elsewhere in Africa and globally. The second IHR Emergency Committee, convened on 18 August 2026, also reviewed the evolving situation and emphasized that the outbreak remains far from controlled, and continues to constitute a Public Health Emergency of International Concern.

Public health response

Coordination

- Joint WHO–Africa CDC–partner coordination continues through the Continental Incident Management Support Team, with increasing emphasis on translating strategic priorities into coordinated field action. Technical assistance, deployments and operational support are being aligned with evolving transmission patterns and priority response gaps.
- At country level, the Government of the Democratic Republic of the Congo recently adopted a revised 180-day multisectoral response plan, providing the operational framework for the next six months. The plan prioritizes surveillance and contact tracing, clinical care, community engagement and risk communication, logistics, and preparedness in at-risk areas, with implementation requirements estimated at US\$1.3 billion.
- Response management is becoming increasingly field-oriented, with personnel, technical expertise and operational resources being directed towards the highest-burden and emerging transmission areas. Stronger links between national, provincial and zonal teams are intended to accelerate information flow, problem-solving and operational decision-making.
- A broad network of partners continues to support implementation across affected and at-risk areas, including UNICEF, International Organization for Migration (IOM), Médecins Sans Frontières (MSF), Alliance for International Medical Action (ALIMA), International Medical Corps (IMC), Samaritan's Purse, the European Union, Global Outbreak Alert and Response Network (GOARN), Emergency Medical Teams and StandBy partners, supporting field operations, laboratories, logistics and specialist deployments.

Operations Support and Logistics (OSL)

- The emergency supply pipeline increased to US\$12.84 million, up from US\$12.18 million (+US\$0.66 million). Supplies worth US\$5.35 million (42%) have been delivered, US\$6.39 million (50%) are in transit, and US\$1.09 million (9%) are not yet ready for shipment.

- Delivery of supplies continued to progress, with cumulative delivered cargo reaching 355.0 metric tonnes (+8.3 tonnes) and 2014.4 m³ (+41.7 m³), compared with 346.7 metric tonnes and 1972.7 m³ in the previous update.
- Critical commodities continued to reach the Ituri and Kivu hubs. Cumulative deliveries include 82 274 hooded coveralls (+4000), 5.53 million examination gloves, 158 800 Level 3 gowns, 261 hospital beds, 10 076 infrared thermometers and 20 016 RadiOne PCR tests. Manual PCR tests increased to 8640 (+1,440). In transit are 250 930 hooded coveralls, 4.19 million examination gloves, 303 000 Level 3 gowns, 500 hospital beds and 8000 infrared thermometers.
- Personal protective equipment / infection prevention and control supplies remain the largest procurement category, increasing to US\$5.32 million from US\$5.17 million. Laboratory supplies increased to US\$2.08 million from US\$1.91 million, while medical equipment reached US\$1.97 million, logistics US\$1.37 million, and medicines remained approximately US\$0.82 million.
- The share of the pipeline delivered or in transit is now 92%, compared with 93% previously, as the value of supplies not yet ready increased to US\$1.09 million (9%) from US\$0.85 million (7%). Procurement remains highly concentrated, with the principal supplier accounting for 82% (US\$10.97 million) of total order value, compared with approximately 83% (US\$10.68 million) previously.

Surveillance

- Surveillance activities continued across affected and at-risk areas, including active case finding, alert management and investigation, contact tracing, mortality surveillance, community-based surveillance, and surveillance at points of entry (PoEs) and points of control (PoCs).
- From 7 – 13 September, a total of 13 709 alerts were reported, averaging 1958 alerts per day, broadly unchanged from 13 730 (1961/day) during 31 August – 6 September. Alert verification declined from 89.4% to 82.9% (11 366 / 13 709). Of 2845 validated suspected cases, 2720 (95.6%) were investigated, compared with 100% in the preceding period. Among those investigated, 1370 (50.4%) were isolated and transferred to treatment centres within 24 hours of investigation, down from 53.2%. The decline across verification, investigation and isolation indicates gaps along the alert-management cascade, particularly in ensuring that investigated suspected cases are promptly isolated and referred.
- Contact-tracing workload increased during 7 – 13 September, with an average of 26 373 contacts requiring daily follow-up, compared with 24 312 during 31 August – 6 September. Average follow-up performance declined slightly from 86.9% to 85.6%, remaining below the ≥95% target. As of 13 September, a total of 27 905 contacts were under follow-up, of whom 21 935 (78.6%) were seen. Performance varied considerably across provinces, ranging from 87.6% in Ituri and 84.7% in Bas-Uélé to 77.9% in Tshopo, 69.2% in Nord-Kivu, 65.7% in Haut-Uélé and 41.0% in Sud-Ubangi. The particularly low follow-up in Nord-Kivu is operationally important given its large contact burden and increasing transmission.
- At points of entry (PoEs) and points of control (PoCs), a total of 252 583 travellers were recorded on 13 September, with 98.1% screened. One alert was detected in Ituri, validated as a suspected case, investigated and transferred to a treatment centre. Overall reporting completeness improved to 88.9%, although important gaps persisted in Sud-Kivu (55.5%) and Haut-Uélé (54.5%).

Laboratory

- Diagnostic capacity continued to expand and decentralize, increasing from two laboratories at the onset of the outbreak to 26 active laboratories. Of these, ten primarily use open RT-PCR platforms and 16 primarily use RadiOne near-point-of-care PCR platforms, providing a reported combined testing capacity of more than 4700 samples per day. New laboratory capacity has been established in Butembo Health Zone with the activation of a Ugandan mobile laboratory. The set up of additional laboratories and diagnostic platforms is planned.
- Laboratory validation of candidate BVD diagnostic tests under the BRIDGE-Access initiative commenced at the Institut National de Recherche Biomédicale (INRB) in Kinshasa on 14 September, supporting the evaluation of BDBV-specific and broader filovirus assays, including near-point-of-care tests.. Results will contribute to generating the clinical evidence needed to inform deployment, procurement, and regulatory pathways for outbreak diagnostics.
- Priorities are now to consolidate and sustain the expanded network through implementation of the updated decentralization plan; reliable supplies, equipment, training and supervision; and improved turnaround-time data capture, specimen management and partner reporting. WHO is procuring approximately US\$1.5 million in laboratory supplies and critical equipment to support new and existing laboratories.

Case Management

- Case-management capacity includes 49 treatment centres/units providing 1390 active beds, with 905 patients hospitalized or in isolation as of 13 September, corresponding to a national bed occupancy of 66.3%.
- Capacity remains uneven across affected provinces, with Nord-Kivu operating at 145.6% occupancy, compared with 68.0% in Sud-Kivu, 51.5% in Haut-Uélé, 46.5% in Ituri and 25.8% in Tshopo. Seven patients remain in isolation in Bas-Uélé, although a dedicated treatment centre is not yet available.
- Clinical capacity and quality of care continue to be strengthened, including expanded diagnostic and critical-care capacity and adaptation of beds to epidemiological and geographical patterns and patient needs.
- Important gaps persist, with several facilities managing suspected cases in Ituri saturated, 45% of hospitalized patients in Nord-Kivu managed outside designated treatment centres, and no beds available in Butembo and Katwa. Tshopo faces limited technical capacity and lack of safe blood, while the absence of a treatment centre/unit and ambulance continues to constrain isolation and referral in Bas-Uélé.
- Cumulative recoveries increased to 1726 as of 13 September 2026, up from 1563 on 6 September, representing 163 additional recoveries over the reporting period.
- A dedicated site to train treatment centre staff was opened and the first training for 30 people successfully completed. The four-day training is comprised of both theoretical and practical sessions in a simulated real-life treatment facility.

Research and Development

- Following the 17 August 2026 allocation of 70 000 doses of Ervebo® to the Democratic Republic of the Congo,

20 000 doses were allocated to the Phase 3 clinical trial, 20,000 to the Bundibugyo Ring-vaccination Assessment of Vaccine Outcomes (BRAVO) observational vaccine-effectiveness study among healthcare workers in Ituri, and 30,000 for use among healthcare and frontline workers under separate research protocols with informed consent.

- Ervebo® vaccination of healthcare and frontline workers continues to expand. In Tshopo, 2307 of 11 703 targeted healthcare and frontline workers (19.7%) had been vaccinated as of 13 September, including 1448 in Makiso-Kisangani, 313 in Mangobo, 312 in Kabondo, 150 in Tshopo and 84 in Lubunga. Bas-Uélé received an additional 500 doses in Buta, while preparations were completed to initiate vaccination of frontline workers in Bunia, Ituri, from 15 September. Uptake remains suboptimal in Tshopo, and the Bafwasende vaccination site is not yet operational.
- Ervebo® use for BVD remains within a research context. Following its 19 August review, the WHO Strategic Advisory Group of Experts on Immunization (SAGE) concluded that available evidence was insufficient to support programmatic use for BVD and that efficacy against BVD in humans remains unknown.
- Development and evaluation of BDBV-specific vaccine candidates continue, including Oxford's ChAdOx1 and Moderna's mRNA candidates in Phase 1 trials, alongside additional rVSV and MVA candidates. Initial results from the Oxford trial are expected by the end of September..
- Therapeutic and prophylaxis research also continues. The WHO-sponsored PARTNERS trial is evaluating MBP134, remdesivir and their combination for confirmed BVD, while the EBO-PEP trial is evaluating post-exposure prophylaxis for high-risk contacts. In Tshopo, 128 people had received investigational treatment or prophylaxis, including 16 confirmed cases and 112 receiving post-exposure prophylaxis. Prophylaxis for high-risk contacts is also ongoing in Bas-Uélé.
- The MBOTE clinical characterization study continues recruitment across four centres to characterize the clinical presentation, progression and outcomes of BVD, and complement evidence generated through ongoing therapeutic studies.

Infection Prevention and Control (IPC) and Water, Sanitation and Hygiene (WASH)

- IPC activities continued across affected provinces, although implementation remained uneven. Targeted IPC interventions around confirmed cases and their affected households and health facilities were initiated for only 41.0% of expected case clusters in Ituri and 40.0% in Haut-Uélé, while only 22.4% of identified affected households were decontaminated in Nord-Kivu.
- Safe and dignified burials continued, with 65 of 77 expected burials (84.4%) completed in Ituri and 53 conducted in Nord-Kivu on 13 September. Gaps in body swabbing persist because of community resistance and shortages of supplies and teams .
- IPC capacity in health facilities remains inadequate in several areas, with only two of 92 facilities (2.2%) in Tshopo achieving an IPC score above 80%, while Bas-Uélé lacks adequate triage and waste-management infrastructure across hotspot facilities.
- Operational constraints remain substantial, including shortages of IPC supplies, limited mobility and

decontamination teams, insufficiently trained personnel, and community resistance in several health zones.

- Healthcare worker infections remain a major concern, with 167 confirmed infections and 47 deaths, underscoring persistent healthcare-associated transmission risks and the need to strengthen triage, PPE use and adherence to IPC standards.

Risk Communication and Community Engagement (RCCE)

- RCCE activities continued across affected provinces, using household visits, educational sessions, mass communication, community dialogues and engagement with religious and community leaders. Activities remained particularly intensive in Nord-Kivu, Ituri and Tshopo, with increasing emphasis on affected communities and groups at higher risk.
- Feedback mechanisms also remained active, with most reported concerns addressed within 48 hours, although completeness of community reporting remains uneven.
- Locally adapted engagement approaches continue to be strengthened, including engagement with religious and community leaders and targeted outreach in affected communities. In Tshopo alone, mass sensitization through religious settings reached more than 27 000 people, while community dialogue and survivor engagement continued in other affected provinces.
- Community resistance and misinformation remain important operational challenges, including rumours and continued difficulties with acceptance of response measures in some affected communities. These are compounded by shortages of communication materials, transport and connectivity in Ituri, Haut-Uélé and Bas-Uélé, reinforcing the need for sustained engagement through trusted local actors and rapid management of community concerns.

Preventing and Responding to Sexual Exploitation, Abuse, and Harassment (PRSEAH)

- PRSEAH community awareness activities expanded across Ituri, Tshopo, Haut-Uélé, Nord-Kivu and Sud-Kivu, reaching 1119 people, including 278 women and 266 girls, during the reporting week. Community leaders, civil society members, teachers and students, and faith communities were engaged to strengthen awareness of sexual exploitation, abuse and harassment risks and available reporting mechanisms.
- PRSEAH capacity-building was further integrated into the response workforce orientation, with 802 people, including 333 women, participating in briefing sessions. Participants included hygienists, community leaders, healthcare providers and personnel working in treatment centres.
- A joint sexual exploitation, abuse and harassment risk assessment is underway in Tshopo, Bas-Uélé and Haut-Uélé, involving the WHO, United Nations Population Fund (UNFPA), United Nations Children's Fund (UNICEF), Mercy Corps and the United Nations Development Programme (UNDP), in coordination with relevant local stakeholders. The assessment aims to identify risks associated with response activities and inform targeted prevention and mitigation measures.
- Coverage of PRSEAH reporting mechanisms continues to expand but remains insufficient, with awareness currently documented in 13 of 36 health zones (36.1%) in Ituri. Strengthening coordination and alignment

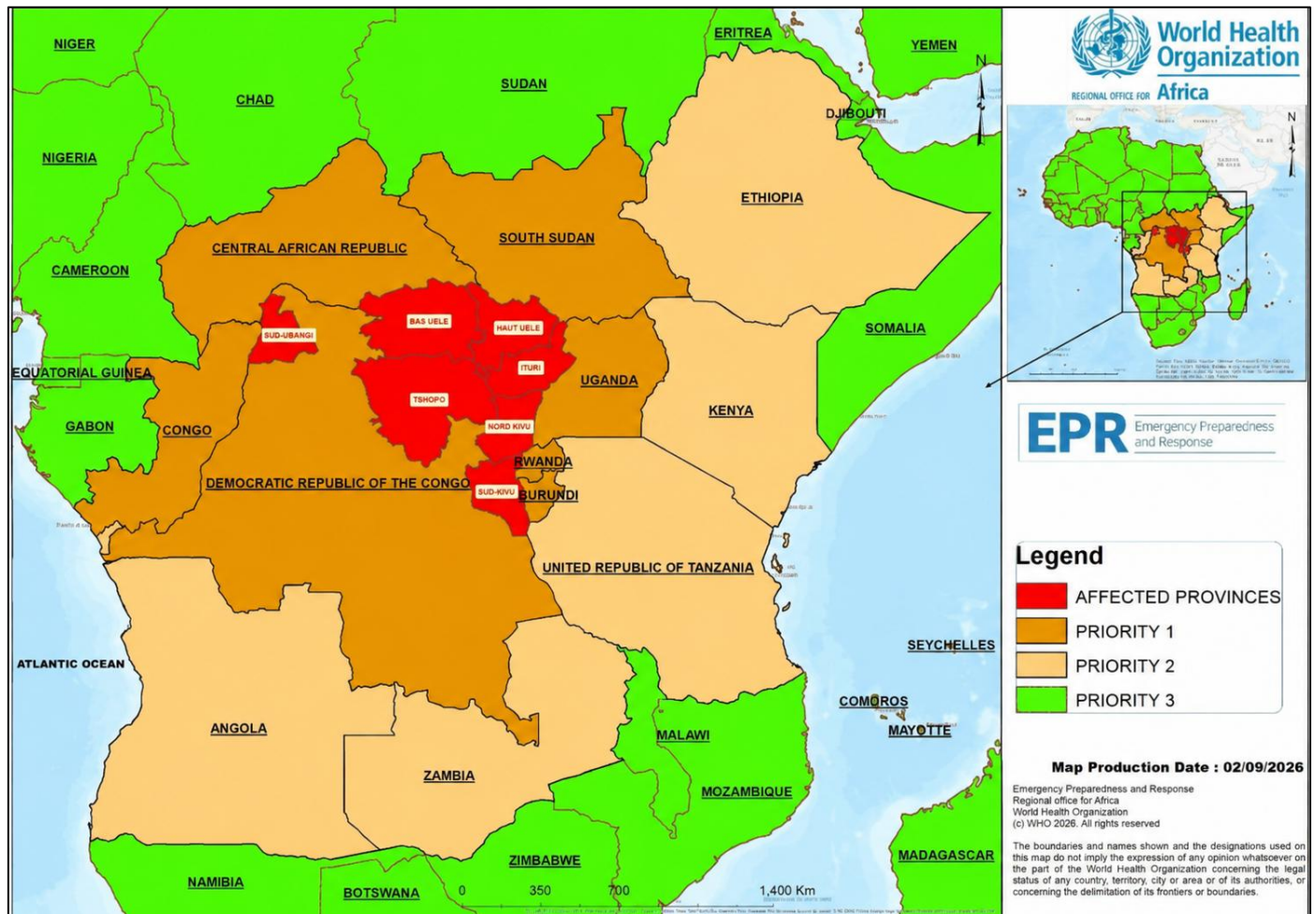
among PRSEAH partners remains a priority to expand coverage, reduce duplication and ensure consistent prevention, reporting and response mechanisms across affected areas.

Public health preparedness

Operational Readiness

- Based on regional cross-border risk modelling and the evolving epidemiological situation, countries have been recategorized by priority for response and preparedness. The Democratic Republic of the Congo is classified as the response country (Priority 1A); Uganda, Central African Republic, South Sudan, Republic of the Congo, Burundi and Rwanda as Priority 1B countries; and Angola, Ethiopia, United Republic of Tanzania, Kenya, and Zambia as Priority 2 countries.

Figure 6. Regional prioritization of countries for Bundibugyo virus disease preparedness and response, as of 13 September 2026



- Ten out of 11 high-priority countries completed repeat readiness assessments, reflecting continued efforts to strengthen national preparedness capacities: Angola, Burundi, Central African Republic, South Sudan, Tanzania, Kenya, Republic of the Congo, Rwanda, Uganda and Zambia, average scores have increased from 55% to 61%.
- Regional technical deployments continued to support preparedness and response capacity in affected and high-risk Member States, including Central African Republic, Democratic Republic of the Congo and Uganda.

Border Health, Travel and Mass Gatherings

- Screening and public health measures have been reinforced at airports, high-risk border crossings, major ports and internal transit routes in the Democratic Republic of the Congo and Uganda. National authorities, IOM, and partners continue to support the implementation of WHO border health guidance, strengthen cross-border coordination, and facilitate the timely detection, reporting and referral of suspected cases at PoEs and PoCs along priority travel corridors.
- Cross-border preparedness and response are being strengthened through enhanced coordination among the Democratic Republic of the Congo, Uganda, the Central African Republic and South Sudan to improve the early detection of cases and reduce the risk of cross-border spread. Priority actions include strengthening coordinated cross-border disease surveillance, event-based surveillance, active case finding, joint investigations and contact tracing where required, and real-time information sharing through regular cross-border coordination meetings and harmonized reporting mechanisms.
- Priority operational actions include rapid assessments and mapping of PoEs, strengthening surveillance along high-risk cross-border mobility corridors, including informal crossings, and major congregation points; and ensuring readiness capacities through the provision of trained personnel, equipment, PPE, referral mechanisms, disinfectants, risk communication materials and contingency plans for the safe management of suspected cases.
- The postponement of mass gatherings is advised in areas with documented BVD transmission until transmission is interrupted. Event-specific risk assessments should be conducted using the WHO all-hazards mass gathering risk assessment tool. These assessments should be complemented by implementation of measures before, during, and after events, as outlined in WHO guidance for mass gatherings attended by individuals from areas with documented Bundibugyo virus detection, including the establishment of referral pathways and RT-PCR testing capacity.
- Event organisers should distribute participant information to all attendees before and during the event, covering BVD symptoms, what to do if feeling unwell, and available health services, in line with [WHO guidance for mass gathering events attended by individuals from areas with documented BDBV detection](#).

Situation interpretation

The outbreak remains uncontrolled and increasingly heterogeneous, with declining transmission in Ituri masking rapid intensification in Nord-Kivu, and continued geographic expansion, including to Sud-Ubangi. Response efforts should prioritize targeted interventions in emerging and persistent hotspots, particularly strengthening early detection, contact tracing, rapid isolation and referral, IPC and community engagement. Approaches contributing to declining transmission in areas of Ituri should be identified, consolidated and rapidly adapted and scaled up in areas where transmission is increasing. Preparedness should also be reinforced in high-risk health zones and along domestic and cross-border corridors to limit further geographic spread.

Contributors

- WILLIAMS, George Sie (WHO AFRO)
- OTSHUDIEMA, John (WHO AFRO)
- CONTEH, Ishata (WHO AFRO)
- MASINA, John (WHO AFRO)
- MAMADU, Ibrahim (WHO AFRO)
- ROQUILLE, Denise (WHO AFRO)
- MUTEBA, Michel (WHO AFRO)
- DOSHI, Reena Hemendra (WCO AFRO)
- KAMBALE KAVOGA, Freddy (WHO AFRO)
- NGUNA, Joyce (WHO AFRO)
- PAVLIN, Boris (WHO HQ)
- HAMBLION, Esther (WHO HQ)
- KASSAMALI, Zyleen (WHO HQ)
- DIAZ, Janet Victoria (WHO HQ)
- ALLAIN IOOS, Sophie (WHO HQ)
- HAMOUDA, Mahmoud Aly Zeenelabdin (WHO HQ)
- MOLINARO, Paul (WHO HQ)
- LEGAND, Anaïs (WHO HQ)
- WAUQUIER, Nadia (WHO HQ)
- SODAGAR, Vaishali (WHO HQ)
- WANG, Ninglan (WHO HQ)
- SELENIC MINET, Dubravka (WHO HQ)
- ALEXANDROVA EZERSKA, Lidia (WHO HQ)
- MORAN, Thomas (WHO HQ)
- FOTSING, Richard (WCO DR Congo)
- AJONG, Brian (WCO DR Congo)

© WHO Regional Office for Africa

This is not an official publication of the World Health Organization.

Correspondence on this publication may be directed to:

Health Emergency Intelligence, Surveillance and Laboratory Programme

WHO Emergency Preparedness and Response

WHO Regional Office for Africa

Email: afrooutbreak@who.int

Subscribe here to receive regular updates via email: [SUBSCRIBE](#)

Editorial Advisory Group, WHO Regional Office for Africa

- Dr Marie Roseline Darnycka Belizaire — Regional Emergency Director, EPR/AFRO
- Dr Otim Patrick Ramadan — Programme Area Manager, EMR/EPR/AFRO & Deputy Regional Incident Manager, BVD Response
- Dr Etien Koua — Programme Area Manager, HIS/EPR/AFRO
- Dr Dick Chamla — Programme Area Manager, HEP/EPR/AFRO
- Mr Adama Thiam — Programme Area Manager, OSL/EPR/AFRO

Requests for permission to reproduce or translate this publication – whether for sale or for noncommercial distribution – should be sent to the same addresses.